

Lesson 19: Ethical Issues in Psychotherapy

1. Informed Consent to Therapy

Informed consent is a crucial ethical and legal obligation. Therapists must ensure clients understand key aspects of their treatment from the start.

- **Standard Informed Consent (Established Techniques):** Psychologists should inform clients as early as feasible about:
 - The **nature and course** of therapy.
 - **Fees**.
 - The involvement of **third parties**.
 - **Limits of confidentiality**.
 - Answers to any related **questions** about therapy.
- **Consent for Non-Established Treatments:** For treatments without generally recognized techniques, therapists must inform clients about:
 - The **developing nature** of the treatment.
 - **Potential risks** involved.
 - **Alternative treatments** if available.
 - The **voluntary nature** of their participation.
- **Therapist-in-Training:** If the therapist is a trainee, the client must be informed that the legal responsibility for treatment lies with a supervisor. The client should be given the supervisor's name during the informed consent procedure.

2. Group Therapy

When providing services to a group, psychologists must clarify the roles and responsibilities of all parties and the limits of confidentiality at the outset.

- **Advantages:**
 - Peer influence and support can facilitate gains that might be slower in individual treatment.
 - The group setting can become a "therapeutic ecology" where special insights and awareness may develop.
 - **Disadvantages & Hazards:**
 - A group leader who lacks proper training may fail to monitor the experience adequately for all members.
 - Pressures for cohesion and emotional expression can be inappropriate for some clients.
 - The therapist has less control over the content and direction of the session compared to individual therapy.
 - Potential problems include stress from confrontation, criticism, threats to confidentiality, and dependency on the group.
 - **Preventing Negative Outcomes:** To manage these risks, a therapist must:
 - Provide **informed consent**.
 - Ensure **voluntary participation**.
 - Conduct proper **screening of participants**.
 - Carefully differentiate roles based on whether the group is **therapeutic or educational**.
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3. Therapy Involving Couples or Families

When working with multiple people in a relationship (e.g., spouses, parents & children), therapists must clarify at the outset:

- **Which individuals are clients.**
- The **relationship the psychologist will have with each person.**
- The psychologist's **role** and the probable uses of the information obtained.

- If potential **conflicting roles** arise (e.g., being asked to witness in a divorce), the psychologist must take reasonable steps to clarify, modify, or withdraw from the role appropriately.
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4. Providing Therapy to Those Served by Others

If a client is already receiving services from another professional, a psychologist must carefully consider treatment issues and the client's welfare. They should discuss these issues with the client to minimize the risk of confusion and conflict.

5. Sexual Intimacies

Psychologists are strictly prohibited from engaging in sexual intimacies with:

- Current therapy clients.
 - Individuals they know to be close relatives or guardians of current clients.
 - Significant others of current clients.
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6. Interruption of Therapy

Psychologists must make reasonable efforts to plan for continuity of care if services are interrupted. This can be due to factors such as:

- The psychologist's illness, death, unavailability, or relocation.
 - The client's relocation or financial limitations.
 - When entering or ending employment/contractual relationships, the psychologist must ensure responsibility for client care is resolved in a timely and appropriate manner.
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7. Special Obligations of the Therapist

- **Client's Frame of Reference:** Therapists must be aware of and take into account the social, economic, and cultural pressures on clients, especially those from marginalized groups.
- **Right to Refuse Treatment:** Clients generally have the right to refuse treatment if they disagree with the therapist's approach or risk-benefit statement.
- **Conflicting Values:** The therapist must avoid imposing their personal values on the client.
- **Difficult Clients:** Therapists may encounter clients who are difficult to work with, such as those who make frequent suicidal threats, are dangerous, fail to show up for appointments, or harass the therapist's family.

Lessons 20 & 21: Therapy Termination I & II

1. Special Obligations of the Therapist

When a client threatens, the therapist has special obligations.

- **Clients who threaten are often:**
 - Facing personal or family distress.
 - Dealing with serious mental illness and impulse control issues.
 - Having problems with anger control.
 - Exhibiting a history of antisocial behavior.
 - **How a psychologist can deal with such situations:**
 - Maintain clinical competence and good diagnostic skills.
 - Understand the confidentiality issues involved.
 - Be mindful of the potential danger.
 - Engage in careful advance planning.
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2. When to Terminate Therapy

Therapists must terminate therapy ethically and legally. According to the APA Ethics Code, termination is appropriate when:

- It is reasonably clear the client **no longer needs the service, is not likely to benefit, or is being harmed** by continued service.
 - The client and therapist have a **major disagreement** on treatment issues.
 - The psychologist is **threatened or endangered** by the client or someone in the client's life.
 - A therapist is simply **not a good fit** for the client, especially as the client's needs change over time.
 - A situation arises that could **negatively affect the therapist's judgment or objectivity** (e.g., an inappropriate secondary relationship).
 - A client **repeatedly no-shows** or presents other practical concerns like unreasonable demands or inability to pay.
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3. Guidelines for Therapy Termination

Termination is a process that should be handled thoughtfully, regardless of the reason.

- **General Guidelines:**
 - The purpose of therapy is to support the client, not the therapist.
 - Don't be defensive or argue with the client, even if they are unhappy.
 - Be compassionate, clear, and direct.
 - Never blame the client.
 - Do not abandon a client without warning.
 - Be willing to answer questions and provide referrals to qualified professionals.

- Leaving the door open for a potential return can help clients feel secure.
 - Address the termination of treatment in advance, in a thoughtful and sensitive manner.
 - **Terminating with an unhappy client:**
 - Listen to the client's feedback, as it may help you become a better therapist.
 - If the client accuses you of wrongdoing, take careful notes and consider writing a termination letter.
 - **Terminating with a child:**
 - Discuss termination at the beginning of therapy and prepare the child in advance.
 - Explain why therapy is ending in age-appropriate terms.
 - Highlight the child's progress.
 - Plan a special termination activity to memorialize the therapy.
 - Discuss strategies for managing emotions and help the child develop a list of supportive people.
 - Discuss termination with the parents as well.
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4. The Termination Letter

A termination letter is a formal way to record the end of therapy and the reasons for it.

- **Purpose:**
 - It helps clarify the nature and reason for termination, especially if a client is emotional or angry.
 - It can offer some protection if a client later claims abandonment.
- **When to send one:**
 - With a client who has difficulty processing rejection.
 - Due to a poor fit between client and therapist.

- If a client repeatedly no-shows, it may be the only way to ethically terminate.
- Some therapists send a brief termination letter to every client who leaves.

Lesson 22: Guidelines for Record Keeping

1. Terminating Therapy with a Child

Termination can be difficult for children, especially those who have experienced loss.

- **Key Strategies:**
 - Discuss termination at the beginning of therapy and prepare the child as far in advance as possible.
 - Explain the reason for termination in age-appropriate terms (e.g., they've made so much progress they no longer need therapy).
 - Talk about termination in the last session.
 - Encourage the child to share their feelings.
 - Plan a termination activity to memorialize the therapy and progress.
 - Help the child develop a list of supportive people they can contact for help.
 - Discuss termination with the parents, identifying strategies to help the child adjust and criteria for returning to therapy.

2. APA Guidelines for Record Keeping

The APA provides guidelines to educate psychologists and provide a framework for professional record-keeping.

- **Benefits of Records:** They document treatment plans, services provided, and client progress, benefiting both the client and the

psychologist.

- **Information in a Client's File:**

- Identifying data (name, ID), contact information, and billing details.
- Documentation of informed consent or assent.
- Presenting complaint and diagnosis.
- Plan for services and health history.

- **For Each Substantive Contact:**

- Date and duration of the session.
- Type of service (e.g., consultation, assessment).
- Nature of the professional intervention (e.g., treatment modalities, referral).
- Formal or informal assessment of the client's status.

- **Other Specific Information to Record:**

- Client responses or reactions to interventions.
 - Current risk factors (dangerousness to self or others).
 - Other treatment modalities used (e.g., medication).
 - Emergency interventions (e.g., hospitalizations).
 - Plans for future interventions and prognosis.
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3. Record-Keeping Principles

- **Responsibility:** Psychologists have a professional and ethical responsibility for maintaining and retaining their records. Records can be the only way for a psychologist to know what they did and why.
- **Content:** Records must be accurate, current, and pertinent. They help ensure good care, assist other professionals, provide for supervision, and ensure continuity of services. Records are also crucial for documentation needed for reimbursement, legal complaints, and high-risk situations.
- **Confidentiality:** Therapy relies on privacy, and a breach of confidentiality can destroy the therapeutic relationship. Psychologists

must take reasonable steps to maintain the confidentiality of information. This is mandated by law, regulation, and ethical standards.

- **Disclosure:** When appropriate, psychologists should inform clients about the nature and extent of record-keeping procedures, including the limits of confidentiality.
- **Organization:** Records should be updated and logically organized to facilitate retrieval and continuity of care in case of the psychologist's death, disability, or a transfer of care.
- **Retention:** Psychologists should be aware of applicable laws and retain records for the required period. Retention is also beneficial for legal disputes.
- **Preserving Context:** Psychologists must be attentive to the situational context in which records are created to prevent misuse or misinterpretation of data that could harm the client.
- **Electronic Records:** Like paper records, these must be created and maintained to protect their security, integrity, and confidentiality. The ease of sharing electronic records can increase the risk of unintended disclosure.
- **Organizational Settings:** Record-keeping requirements in settings like hospitals or schools may differ from private practice, and psychologists must follow the organization's policies while also adhering to the APA Ethics Code.
- **Multiple Client Records:** In couple, family, or group therapy, psychologists must carefully consider documentation procedures to respect the privacy and confidentiality of all parties. Disclosing information about one client may compromise the confidentiality of others.

Lesson 23: Access to Records & Personal Information

1. Financial Records

Psychologists must ensure their financial records are accurate and complete. This helps with billing, managing payment obligations, and alerting both the psychologist and the client to accumulating balances that could harm the professional relationship.

2. Disposition of Records

Psychologists must plan for the transfer and disposal of records to ensure continuity of treatment and appropriate access.

- Client records require special handling during transitions like job changes, relocation, or death.
 - Methods used for disposal must preserve confidentiality and prevent the recovery of information.
 - Under legal and regulatory requirements, a psychologist may need to notify clients about changes in the custody of their records.
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3. Access to Records & Personal Information

Mental health professionals keep records for legal obligations, as a memory aid, for communication with other professionals, and to document services. These records often contain confidential material, and various parties may seek access to them.

- **Key considerations for psychologists:**
 - Securing a client's **informed consent** for the release of information.
 - Considering the claims and circumstances of those seeking access.
 - The use of records for **teaching or research** purposes.
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4. Consent for the Release of Records

Under HIPAA regulations, a consent or release form must contain specific details:

- A clear description of the information to be disclosed.
 - The name of the person(s) authorized to disclose and receive the information.
 - The purpose of the disclosure.
 - An expiration date or event related to the purpose.
 - The signature of the person authorizing the release and the date.
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5. Client's Access to Records

Clients' access to their records has been a controversial issue. Therapists should assume any client may ask to see their records and will ultimately be able to obtain copies. Under HIPAA, there are four special categories of records:

- **Medical Records:** General office records including appointments, diagnoses, prescriptions, and insurance claims.
 - **Psychotherapy Notes:** Observations recorded for the therapist's own use, containing details beyond standard medical records. These must be stored in a separate file from medical records.
 - **Forensic Reports:** Reports written for legal contexts, such as competency evaluations. Their purpose is legal, and they may be governed by court rules or orders.
 - **Working Notes:** Impressions, hypotheses, or half-formed ideas a professional jots down to assist in formulating reports. These are speculative and temporary, and may not have meaning or utility to anyone else.
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6. Access to Records by Family Members

- Therapists must be sensitive to each individual's right to privacy, especially when working with minors or families.
- When a client is a child or legally incompetent, parents or guardians generally have full legal entitlement to record access.
- For adult clients, access to records by family members should generally be denied unless there is a special reason, such as a serious and imminent threat of harm to self or others, or legally adjudicated incapacity.
- In a difficult case where a client cannot speak for themselves, it may not be unethical for a therapist to respond to a duly authorized request for information from the next of kin.

Lesson 24: Access to Records & Personal Information II

1. Issues in Access to Child's Record

Therapeutic confidentiality is crucial for effective treatment, even for minors. However, a parent or guardian is generally the minor child's personal representative, and a healthcare provider is permitted to share a client's information with their personal representative under the Privacy Rule.

- **Why Confidentiality is Important for Children:**
 - It increases cooperation in treatment, as children may not disclose important information if they fear it will be shared with their parents.
 - It ensures a child gets effective treatment by providing the therapist with enough information to understand their needs.
 - It protects the child from the risk of abuse or homelessness, such as in cases where parents may abuse a child for their sexual orientation.

- It protects the child from third parties who might use confidential information for bullying or identity theft.
 - It can improve the parent-child relationship by allowing the child to openly discuss feelings in therapy.
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2. Exceptions to Parental Access

A parent is not treated as a minor child's personal representative in several important situations:

- When state law does not require parental consent for a specific health service.
 - If the minor child consents to the service and has not requested the parent be treated as a representative.
 - If a parent agrees to a confidential relationship between the minor and the therapist.
 - In contentious custody cases, a court may appoint a lawyer (guardian ad litem) who determines if and when parents can view treatment information.
 - When a court order specifically prohibits the parent from accessing the child's information.
 - If a parent has lost or given up their parental rights, such as in the case of adoption.
 - When the child is emancipated, meaning they are legally responsible for their own care before the age of 18.
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3. Discussing Confidentiality with Youth

Therapists have an important ethical duty to discuss confidentiality with both the child and their parent(s).

- **Key topics to discuss include:**

- The therapist's disclosure policies and any specific confidentiality agreements they require parents to consent to.
 - The specific circumstances under which a therapist would disclose information shared in therapy.
 - A HIPAA disclosure statement that details how and when treatment information may be disclosed to others.
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4. Legal Protections for Minors

The right to privacy is connected to a client's right to consent to treatment. Since a child cannot legally consent, a parent often acts as their personal representative.

- Parents generally have the right to request their child's medical record, including diagnosis, symptoms, and treatment plan.
 - However, a parent does not have the right to view **psychotherapy notes** unless a court orders it. These notes are for the therapist's personal use and analysis, not official documentation.
 - The HIPAA Privacy Rule requires therapists to take reasonable steps to protect client privacy and protects minors from disclosures to third parties who are not their parents.
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5. Breach of Confidentiality (Example)

In some crucial cases, a therapist may decide it's necessary to share records with parents, even if it breaches confidentiality, after a critical analysis of the situation and the potential harms of not sharing the information.

- **Case Study: Max**
 - A 7-year-old boy named Max, upset about a new sibling, threatens to kill the baby.

- The therapist must critically assess the seriousness of the threat, considering factors like Max's history of violence.
- If the therapist cannot stop Max from hurting his sibling, they must discuss the matter with his parents as a "duty-to-protect" issue. Not doing so would be considered malpractice.

Lesson 25: Access to Records & Personal Information III

1. Court Access to Records

Courts or litigants may seek access to privileged and confidential information, and mental health professionals must respect appropriate requests from the courts. It's crucial to understand the difference between a subpoena and a court order.

- **Subpoena:** This compels a response, typically requiring you to appear in court to testify as a witness.
 - **Court Order:** This follows a hearing before a judge and compels disclosure unless appealed.
 - **Protection:** Therapists must reasonably safeguard client records and materials from inappropriate release.
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2. Confidentiality After Death

A mental health professional has a duty to maintain a client's confidentiality even after their death.

- Therapy is based on privacy and secrecy, and a breach of confidentiality destroys the therapeutic relationship.
- Post-mortem confidentiality protects clients from defamation and considers the welfare of their survivors.

- A therapist must make a reasonable assessment of when and where it is appropriate to maintain confidentiality.
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3. Records & Cyber-Confidentiality

Changes in how we store and transmit data raise new confidentiality concerns.

- **Hazards:** The vast amount of information stored on small electronic devices can be misused, stolen, or misplaced. Using the internet for communications also presents risks.
 - **Safeguards:** Therapists must be thoughtful and cautious. Special attention should be given to creating strong passwords, backing up data, and using encryption. Locking codes for devices and auto-locking features for computers are also helpful.
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4. Issues in Electronic Record Keeping

With electronic records, there is a risk of errors and conversion problems.

- **Guidelines:**
 - Avoid entering or repeating incorrect information.
 - Check for conversion problems when records are transferred to a new system.
 - Have a plan to access data if the system becomes unusable.
 - Be careful when using prefilled forms or copying and pasting text.
 - Ensure the new system's design aligns with client clinical needs.
 - Guard against electronic routing failures.
 - Inform clients about the nature and security of the system, including who has access to their records.
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5. Third-Party Access: Insurers & Managed Care

When a client submits a claim to an insurance company, certain information is shared.

- **Shared Information:** This can include the client's diagnosis, the type of service, and the dates and duration of treatment.
 - **Risks:** Once information leaves the practitioner's office, it is beyond their control. Insurance companies may not exercise the same caution as the therapist.
 - **Therapist's Role:** The therapist should inform clients about what information will be shared with the insurance company and the associated privacy policies. Clients can also choose to pay out of pocket to avoid using insurance altogether.
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6. Accessing Client's Information on the Internet

There are no specific ethics codes addressing how therapists should obtain information about clients from the internet.

- **Consequences:** Therapists must be prepared to deal with the consequences of information they discover that was not provided by the client.
- **Ethical Challenge:** The key ethical challenge is how a therapist handles a client's data that they may discover from the internet.

Lesson 26: Confidential Information

1. Taking Advantage of Confidential Information

Psychotherapists may occasionally have the opportunity to personally gain from information received in confidence during a therapy session.

- **Ethical Misconduct:** It is unethical to use confidential information for personal gain, especially when it is not public knowledge. A therapist who learned of business events related to the stock exchange from a client and made strategic investments was prosecuted for "insider trading".
 - **Acceptable Use:** The use of information not related to the client's mental health or services, such as a client complaining about an unreliable car mechanic, is generally not considered ethical misconduct because this type of information is widely available.
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2. Confidential Material in Teaching

When using confidential or sensitive material for teaching purposes, the client or their legal guardian should ideally provide full informed consent.

- **Consent is Necessary:** Consent is required when adapting videotapes, audiotapes, or detailed case summaries, especially if the material could make it possible to identify the client.
 - **Disguising Material:** Formal consent may not be necessary if the material is disguised so that the client cannot be identified. However, the therapist should be cautious, as the consequences of revealing confidential details in a recognizable way can be devastating. A good practice is to review the material with a colleague to ensure no identifying facts have been overlooked.
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3. Confidential Material in Research

While confidential research data can sometimes provide enormous social benefits, such as tracking long-term health impacts, careless handling can create problems for both the client and the therapist.

- **Breach Risks:** The careless handling of research data can lead to a breach of confidentiality and put people at risk. In studies involving criminals, reporting a crime to the police could even expose the researcher to retaliation.
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4. Guidelines for Using Confidential Material

To protect client privacy when including case studies or other material in public statements, teaching, or research, therapists should remove or alter specific identifiers.

- **Identifiers to Remove/Alter:**
 - Names.
 - Geographic subdivisions smaller than a state (e.g., specific city).
 - Any dates directly related to an individual (except years).
 - Telephone, fax, Social Security, and other identification numbers.
 - Email and IP addresses.
 - Biometric identifiers and full-face photographs.
 - Any other unique identifying number or code.

Lesson 27: Marketing Professional Services

1. Challenges in Marketing

Advertising mental health services is challenging because the "product" is difficult to characterize. The APA ethics code provides mandates to guide ethical practice in advertising.

- **Ethical Advertising Rules:**
 - Avoid false or deceptive public statements about your practice, research, or credentials.

- Maintain the integrity of statements made by others on your behalf. Advertisements must be identified as such, and you are responsible for those you hire to promote your work.
 - Do not compensate media personnel for publicity in news items.
 - Do not ask current therapy clients or vulnerable individuals for testimonials.
 - When offering public advice (e.g., on the Internet), clarify the scientific basis of the advice and your professional role.
 - Do not engage in unwanted, in-person solicitation of business from vulnerable individuals.
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2. Modes of Advertisement

Therapists use various methods to advertise their services.

- **Older Standbys:** These include listings in telephone directories and business cards.
 - **Direct Solicitations:** While announcing general availability is acceptable, direct solicitation of individual clients has a high potential for abuse and distress due to the therapist's position of influence.
 - **Referral Services:** Professional associations may operate services that provide callers with a list of qualified practitioners who meet their needs.
 - **Internet Advertising:**
 - **Websites:** An ethical website should be simple, easy to navigate, and contain truthful, current information on services, fees, and other relevant data.
 - **Social Media:** When using social media for marketing, be mindful that what you post is visible to everyone. It's important to maintain professional decorum and avoid blurring boundaries with clients.
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3. Important Issues in Advertisement

Therapists should consider several recommendations before advertising.

- Consult with colleagues to understand community standards for advertising.
- Do not delegate the details of your advertising to others, especially those with aggressive marketing strategies and a poor understanding of ethics.
- Proofread or carefully monitor the final product before distribution.
- Retain a copy of the advertisement for documentation in case questions arise later.
- The advertisement should be welcoming to all, including marginalized communities, irrespective of race, religion, or other factors.
- Do not advertise services outside your area of competence. For example, a therapist should not advertise as a trauma-informed therapist without proper training.
- Focus your advertisements on your target audience. For instance, an ad for postpartum depression would be more relevant on a parenting website.

Lesson 28: Mental Health Professionals & Media

1. Ethical Issues in Media Interaction

Mental health professionals must consider several ethical issues when interacting with the media, including social responsibility, competence, conflicts of interest, and the public image of the helping professions. They have a social responsibility to disseminate relevant, educational information.

- **Misinformation:** Misguidance can be unintentional, such as when journalists edit interviews or misinterpret data. It can also be intentional, for example, when authors or publishers compromise a sober presentation of facts to sell more books.

- **Distortions:** Media often portray successful therapy through dramatic emotional breakthroughs, which misrepresents the actual process. Journalists may also select isolated or minor research findings based on curiosity rather than scientific quality, giving a false impression of what was discovered.
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2. Interaction with Mass Media

Appearing on television, writing articles, or being interviewed by journalists raises many professional issues.

- **Interviews with Reporters:** Journalists have control over what information reaches the public, often using small, selected segments that can alter the original message. A mental health professional could be held accountable for inappropriate disclosures to a journalist. It's advisable to ask the journalist questions beforehand to avoid such scenarios.
 - **Guest Appearances:** Live interviews on TV or radio can be positive and educational, but there's a risk of being cut off before a point is fully made or being misidentified (e.g., as a psychiatrist when you are a psychologist).
 - **Popular Publications:** Self-help books written by mental health practitioners are popular, but they don't provide a support system for consumers to correct errors or alleviate negative consequences that may arise from following the program.
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3. Socially Responsible Public Acts

Mental health professionals have a social responsibility to advocate for individuals with mental health problems.

- **Advocacy:** People with a psychiatric history often face discrimination, fear, and mistrust, which limits their opportunities. Professionals are in

the best position to replace misinformation with solid data and challenge inaccurate stereotypes. This includes monitoring negative portrayals and advocating for mental health services.

- **Public Disclosure:** Psychologists may face difficult choices when they observe an unlawful or immoral act but lack the capacity to intervene on their own. The overall spirit of ethics codes prioritizes the well-being of consumers and making positive contributions to society.

Lesson 29: Money and Managed Care

1. The Mental Health Business

Managed care is a broad system that aims to manage healthcare costs, quality, and access to services. It has made earning a living in private practice more challenging. Digital mental health is also becoming a significant factor, extending beyond traditional talk therapy and medication management.

2. Important Issues in the Mental Health Business

Determining what to charge for services is a complex task that involves economics, business, self-esteem, and cultural taboos. Complaints and ethical difficulties related to billing often arise from miscommunications or a lack of knowledge rather than greed.

- **Transparency is Key:** From the start of a professional relationship, a therapist should clearly explain:
 - The nature of the services offered.
 - The fees charged.
 - The mode of payment.
 - Any other financial arrangements.

- **Referrals for Affordability:** If a prospective client cannot reasonably afford the services, the therapist should be prepared to make a sensitive and appropriate referral to more affordable resources like hospitals, community mental health centers, or training programs. It is critical to address these issues early in the relationship to prevent anxiety from taking precedence over other therapeutic concerns.
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3. Fee Splitting

Fee splitting, also known as a kickback, is a practice where a portion of the payment for a service is returned or paid out based on a prearranged agreement. For example, a pre-licensed therapist might be paid 40% of their client fees while the employer keeps 60%.

- **Ethical Concerns:** This practice is traditionally considered unethical because it can:
 - Inhibit appropriate referrals in the client's best interests.
 - Result in unneeded services.
 - Lead to increased service costs.
 - Exploit the relative ignorance of the client.
- **Continuum of Agreements:** A range of fee-sharing arrangements exists, from ethical employer-employee relationships to clearly inappropriate scenarios where a person gets money solely for sending business to another.

Lesson 30: Group Practice

1. What is a Group Practice?

A group practice is a type of therapy business that employs multiple therapists with varying credentials and qualifications. This is in contrast to a private practice, which typically involves a single therapist. Many mental

health providers in a group practice share costs such as rent, secretarial services, and utilities.

2. Advantages of Group Practice

- **Financial Benefits:** Group practices may have higher income potential because operating expenses and administrative costs are shared. Therapists working in a group often don't have to handle their own billing, which allows them more time to spend with clients, increasing their potential client load.
 - **Coworkers:** For therapists who find private practice lonely, a group setting offers professional interaction. Colleagues can also cover missed sessions due to illness, which is helpful for clients who won't have to find an unfamiliar location or therapist when they are already stressed.
 - **Shared Business Responsibility:** A group practice is a good fit for therapists who have little interest in managing the business side of a practice, such as taxes, insurance, and legal issues. Business responsibilities can be shared among colleagues based on individual strengths and interests.
 - **Internal Referrals:** Having a pool of qualified professionals in the same office makes it easy to provide referrals. Instead of terminating therapy and sending a client to an outside professional, a therapist can recommend a colleague in the same practice.
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3. Drawbacks of Group Practice

- **Less Control:** Therapists in a group practice may have little control over aspects of the practice beyond their own sessions. They may also have less outlet to choose their own clients and may find the work overly routine.
- **Financial Issues:** While it's possible to earn a higher income, this isn't always the case due to various factors affecting revenue and profit.

Conflicting issues can also arise, such as when compensation is paid for internal referrals or when percentages of gross income are charged without a legitimate basis.

- **Referral Bias:** There is a tendency to refer clients to therapists one knows. When making a referral, a therapist must offer the client a choice that includes at least one clinician with no connection to the referring party.
- **Office Politics:** Although less of an issue than in other settings, office politics can still present challenges for those who prefer to avoid them. This can be particularly problematic when one therapist has a position of power, such as being the practice owner.

Lesson 31: Referrals

1. Making Referrals

Making appropriate referrals is a key part of a therapist's professional conduct. Referrals should be made thoughtfully, with the client's needs and resources as the top priority. The goal is to provide a seamless transition from one professional to another.

2. Reasons for Onward Referrals

Making a referral is the ethical decision when working within one's own competence is no longer possible. A referral may be necessary if:

- The therapist feels **unsafe** with the client, especially in a private practice setting, and better clinical facilities are needed.
- The client needs help with **other issues** like housing, financial difficulties, or legal matters before they are ready for counseling.
- The client has a **mental-health issue** that prevents the establishment of psychological contact, such as delusions or auditory hallucinations.

- The client's issue is **outside the therapist's area of competence** (e.g., a substance misuse issue that requires a specialist).
 - The number of sessions available is **insufficient** to work safely with the client.
 - The therapist's **modality is not a good match** for the client's issue.
 - The client's difficulties **mirror those of the therapist** (i.e., a parallel process is occurring).
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3. Steps in Making a Referral

Before a therapist tells a client about a specific referral plan, they should ensure a few important things:

- They know **where they will refer** the client.
 - They have ascertained the **current waiting times** for the new provider.
 - They are familiar with the **referral pathways and documentation** needed.
 - If they need to transfer personal information, they must do so ethically, ensuring they get the **client's written consent** beforehand, unless a situation justifies breaking confidentiality (e.g., imminent risk of harm).
 - Even if unable to help fully, the therapist's willingness to **offer support while the client waits** for the new provider can be invaluable.
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4. Important Issues in Making Referrals

- **Prioritizing the Client:** Therapists who provide skillful referrals prioritize their clients' well-being and find them the specialized care they need.
- **Avoiding Rejection:** Clients should not be taken by surprise by a referral, as this could feel like a rejection.
- **Preparation:** The idea of a potential referral should be introduced during the **contracting phase**, before counseling begins. This helps clients

understand that referral is a helpful action, not a threat, and is intended to ensure they get the right help at the right time.

- **Consulting with Supervisors:** When considering a referral, it is always important to discuss the possibility with supervisors. They can help determine if it is the right course of action, especially if the therapist is thinking of a referral due to a lack of confidence.

Lesson 32: Mental Health Professionals in the Legal System

1. Introduction to Forensic Psychology

Mental health and behavioral science professionals are increasingly involved in the legal system as both experts and defendants. Forensic clinicians, specifically, act as expert witnesses and consultants in various legal matters. This area of practice is often considered philosophically distinct from traditional clinical work.

- **How Clients Arrive:** Litigants typically come for mental health evaluations through one of three routes:
 1. **Court Order:** The court mandates the evaluation (e.g., to assess a defendant's competence to stand trial or to offer guidance in child custody cases).
 2. **Lawyer Referral:** The client's lawyer requests an assessment of their mental or emotional state for a potential legal claim or defense (e.g., an insanity defense).
 3. **Informal Agreement:** Both parties in a civil or criminal case agree to an evaluation by the opposing side's experts.
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2. Roles of a Forensic Psychologist

The term "forensic psychologist" covers a variety of roles across different settings. These roles include, but are not limited to:

- **Trial Consultant:** Assists legal professionals with case preparation, jury selection, and witness preparation for both civil and criminal cases.
 - **Expert Witness:** Testifies in court about specialized knowledge, often after evaluating a defendant's mental state. They may also testify on broader psychological principles.
 - **Evaluator:** Assesses criminal defendants or parties in civil litigation on mental health issues relevant to a legal matter. They are typically trained and licensed clinical psychologists with a forensic specialization.
 - **Treatment Provider:** Offers psychological treatment to individuals in settings like forensic hospitals, state psychiatric hospitals, or private practice.
 - **Researcher:** Designs and implements research on forensic psychology and legal issues, often working in universities or research institutes.
 - **Academic Psychologist:** Teaches, trains, and supervises students in higher education settings.
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3. Specialty Guidelines for Forensic Psychologists

Because forensic psychology differs from traditional practice, the

"Specialty Guidelines for Forensic Psychologists" were developed in 1991. The goals of these guidelines are to:

- Improve the quality of forensic psychological services.
- Enhance the practice and systematic development of forensic psychology.
- Encourage a high level of professional practice.
- Encourage practitioners to respect the rights of those they serve.

The guidelines focus on several key areas:

- **Responsibility:** Forensic psychologists must provide services consistent with the highest standards of their profession and ensure their work is used responsibly.
- **Competence:** They should only provide services in areas where they have specialized knowledge and present proof of their qualifications to the court. They must also recognize when their personal values or relationships might interfere with their ability to practice competently. They are responsible for understanding the legal and professional standards that govern their participation in legal proceedings.
- **Relationships:** During the initial consultation, they must inform the legal representative about factors that could affect the decision to contract, such as fee structures, potential conflicts of interest, and the limits of their competence.
- **Confidentiality and Privilege:** They must be aware of legal standards that may limit confidentiality and respect those rights and privileges. They must maintain a system of record-keeping that safeguards a client's privilege and only release information with a court order or the client's consent.
- **Methods and Procedures:** They must stay current with scientific and legal developments and use that knowledge to select appropriate data collection methods for their evaluations, treatments, or research.
- **Public and Professional Communications:** They should ensure their communications and testimony are presented in a way that promotes understanding and avoids deception.

Lesson 33: Mental Health Professionals in Academia

1. The Role of the Educator

The role of a psychology professor or mental health professional in an academic setting goes beyond simply transmitting information. They serve as coaches, supervisors, mentors, and role models for students and clinical interns. Their purpose is to enrich and extend knowledge, and their expertise and passion can inspire students to become competent in their field. Academic institutions with a strong track record typically support academic freedom, allowing for a wide range of course content, teaching styles, and creative assignments. To enhance practical knowledge, courses should include interesting discussions, psychodramas, and role-plays, not just book reading and lectures.

2. Complaints in the Classroom

Academic settings are not without conflict. Both teachers and students have complaints about each other.

- **Teachers' Complaints about Students:** A common complaint from instructors is the irresponsible attitude of their students. They may feel frustrated when their carefully planned lectures are met with indifference, leading to students being unable to grasp concepts or engage critically.
 - **Students' Complaints about Teachers:** Students often complain about:
 - Uninspiring or difficult-to-understand teachers.
 - Unclear grading criteria.
 - Teachers who are unavailable for assistance.
 - Arrogant and demeaning instructors.
 - Teachers who demand too much or assign irrelevant work.
-

3. Competency Issues in Teaching

Effective teaching requires specific competencies, including mastery of the subject matter, proficiency in teaching methods, and impartial student evaluation.

- **Teaching Skills and Course Preparation:** Educators have an ethical mandate to stay current in their field, continuously updating their knowledge through courses, reading, and professional meetings. For example, the field of psychopathology has seen quick advancements with the shift from DSM-IV to DSM-5.
 - **Infusing Diversity Content:** Educators must possess and teach cultural competence, especially in an increasingly globalized world. This involves incorporating diversity into course topics, activities, and assessment criteria.
 - **Teaching without Formal Training:** It is a challenge for instructors to teach courses for which they have little or no formal training.
 - **Impaired Instructors:** Mental health professionals who teach can also be impaired, and students often recognize this, responding accordingly.
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4. Evaluating Student Performance

Evaluation is the process of judging the quality of student work based on established criteria and assigning a value to represent that quality.

Educators have a duty to provide fair and reliable assessments.

- **Doing No Harm:** While most ethics codes mandate doing no harm, educators are sometimes in a position where their duty is to inflict what may be perceived as harmful, such as issuing a failing grade or a critical evaluation. It is important for teachers to maintain a professional decorum during this process.
- **Valid Assessment Strategies:** To ensure valid and reliable assessment, teachers must use strategies that:
 - Are varied and administered over time to demonstrate the full range of a student's learning.
 - Are appropriate for the learning activities, instruction purposes, and student needs.

- Are clearly communicated to students and parents at the beginning of the course.
 - Provide clear directions for improvement.
 - Promote students' ability to self-assess and set goals.
 - Are fair and transparent to all students.
-

5. Respect for Student Privacy

Confidentiality rules in academic settings differ from those in psychotherapy. Careless disclosures can harm students. Instructors should take steps to protect student privacy:

- Do not share students' faces or names without explicit consent.
 - Do not make grades or educational records public. The Family Educational Rights and Privacy Act (FERPA) casts a wide net over this.
 - Be mindful of file names that could disclose student information.
 - Be careful when commercializing the classroom, as posts about educational products could make students nonconsensual spokespeople.
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6. Academic Advisement

Academic advisement is a relationship between an advisor and a student to support educational and career success. A successful relationship requires both the advisor and advisee to understand their responsibilities.

- **Advisor's Roles:**
 - **Take Ownership:** Be knowledgeable about the field and guide students on educational and career goals based on their abilities and interests. A good advisor recognizes their power to intervene and advocate for their students.

- **Short-Range Program Planning:** Advise on course planning, registration, and scheduling adjustments.
- **Long-Range Planning:** Provide consultation on educational and professional objectives consistent with the student's interests and academic abilities.
- **Know When to Refer:** Acknowledge personal limits and be aware of campus and community resources to make effective referrals.

Lesson 34: Ethical Challenges in Work Settings

1. Challenging Workplaces

Mental health professionals are finding employment in an increasing number of non-traditional settings, beyond just clinics or hospitals. The need for psychologists is now felt in fields like law, industry, and policing, for tasks such as hiring, mentoring, and personnel assessment. Ethical pressures in these new workplaces can range from subtle erosions of professional values to overwhelming emotional distress. What's considered the "right" outcome for one person might be the "worst" for another.

- **Example 1: Video Game Evaluation** A psychologist is hired by a computer media company to evaluate children's reactions to video game prototypes that use graphic violence and addictive strategies. The psychologist may be criticized by the public for infusing bad habits in children, even though they are just performing their job.
 - **Example 2: School Competition** A teacher announces that the top student will be celebrated for the best performance on a quiz. This can be a positive motivator for high-achieving students but could be distressing for low-achievers who may feel they can never succeed.
-

2. Ethical Challenges in Specific Settings

- **Government Employment** Working for the government, whether at the municipal, state, or federal level, requires upholding public trust while potentially facing high levels of political pressure. It can be rewarding but also frustrating, especially when integrating professional judgment with policy-making.
 - **Case Example:** A psychologist named Mr. Sad was hired by a municipal government to explore if racist attitudes influenced hiring practices. His analysis confirmed racial discrimination, but officials ordered him to keep the findings confidential. Mr. Sad could not disclose the results on his own due to the ethical code of conduct, which protects the employer's proprietary rights. This highlights the importance of a consultant articulating the nature of their role and expectations upfront.
- **Armed Services** Mental health professionals in the military perform both clinical and non-clinical roles. The non-clinical roles include training, personnel selection and assessment, and research on health-related issues like sleep deprivation and fatigue.
 - **Confidentiality Issues:** Confidentiality is a key concern in military mental health services. To address this, clients must be informed from the outset about all limitations on their confidentiality, such as their profiles being shared with military headquarters. This obligation applies in both military and civilian settings.

Lesson 35: Ethical Issues in Education & Criminal Justice System

1. Ethical Issues in Educational Settings

Psychologists in schools confront several ethical issues, particularly regarding informed consent, privacy, and competence.

- **Parental Consent and Involvement:** Obtaining parental consent for a child's assessment or counseling can be challenging. State laws often grant parents access to a child's records and control over whether they receive special services. For example, a school psychologist might identify a child with criminal tendencies, but the parents may not consent to counseling, making it difficult for the psychologist to provide needed services.
 - **Pressure to Control Costs:** In public schools, there's often pressure to complete student assessments within a limited time frame. This can force psychologists to compromise on the quality of their assessments, as detailed evaluations cannot be done quickly. This pressure can also come from administrators without credentials who attempt to modify professional standards to meet institutional needs.
 - **Dealing with Issues in Education:** Educational institutions with clinical and training components should have policies that ensure competent treatment, adequate supervision, appropriate privacy, and a careful balance of trainee and client needs.
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2. Ethical Issues in the Criminal Justice System

Mental health practitioners in correctional settings perform a wide range of duties, from evaluating and classifying inmates to providing psychotherapy and crisis intervention. The key ethical issue is the degree to which a professional's ethical standards might be compromised by the correctional setting.

- **Conflicting Loyalties:** A correctional counselor may face a conflict between their duty to protect their client's welfare and a duty to warn potential victims. For example, an inmate might reveal a planned escape attempt involving hostages and beg the psychologist not to tell anyone.
- **Dealing with Ethical Conflicts:** To manage such issues, a mental health practitioner should:

- Clarify their professional and legal obligations with prison authorities.
 - Define the limits of their role and the nature of their relationship with the client.
 - Never surrender their professional integrity to competing pressures of the work site.
-

3. Ethical Issues in Community Agencies

Community agencies, which are often government-funded or nonprofit, must balance competing demands from various interests. Mental health professionals in these settings are subject to multiple demands that can lead to significant ethical issues.

- **Competence and Service Demands:** Agencies may be required to offer a wide range of services to secure funding, which can be challenging for a psychologist whose competence is limited to specific issues. This places substantial pressure on the value systems of the practitioners.
- **Home-Based Services:** Mental health professionals providing outreach services, particularly home-based ones, may encounter unusual issues. There may also be a shift away from long-term psychotherapy toward more cost-effective treatment options.

Lesson 36: Ethical Issues in Business & Medical Settings

1. Ethical Issues in Business & Industry

Mental health professionals are often involved as consultants or employees in business settings. Their roles can include:

- **Management consulting and personnel selection:** Hiring mental health professionals helps companies reduce employee turnover.
- **Organizational research:** Industrial/organizational psychologists conduct research on products, consumer buying behavior, and the psychology of consumers.
- **Program evaluation:** They evaluate programs and can assist in designing advertisements to generate a positive consumer response.
- **Training:** They design programs to advance employees' skills and expertise.
- **Advertising and public relations:** They analyze which product features and visuals are more attractive to consumers and help a brand become more popular through customer service policies.
- **Marketing studies:** They determine when certain products should be advertised.

Ethical Challenges ⚠️ The ethical difficulties in business settings stem from the demands of their specific roles and the potential conflict between professional ethics and business ethics. For example, a psychologist may face an ethical dilemma when asked to advertise a controversial product like cigarettes, which are hazardous to health but important for the business's viewpoint.

A psychologist might also feel frustrated when their work is not acted upon, even if the company is satisfied and pays them. In such a case, the psychologist has an obligation to respect the proprietary rights of their employer as long as it's possible to do so while maintaining ethical practice standards.

2. Ethical Issues in Medical Settings

Mental health professionals in medical settings, such as clinical psychologists and health psychologists, are often employed under the

supervision of physicians or in a separate department.

- **Interdisciplinary Collaboration:** Successful diagnosis and treatment of complicated patients often requires a close, collaborative relationship between social, psychological, and medical care providers. For instance, a health psychologist may help a patient with an ulcer identify the source of their stress to aid in their treatment. This integrative approach is necessary for many cases, such as eating disorders, which are often referred to a psychologist after a patient initially presents with physical symptoms.
- **Maintaining Competence:** Practitioners in medical settings must be keenly aware of their expertise and its limitations. Their competency is enhanced through education, experience, and practice.
- **Dealing with Issues:** To deal with these issues, professionals must understand the appropriate role for their services and be prepared to uphold professional standards.

Lesson 37: Independent Practice

1. The Independent Practitioner

Independent practice is becoming more common than working in larger agencies or hospitals. This allows clients to have a one-on-one interaction with a therapist without the involvement of other agencies. An independent practitioner is their own boss but must also handle overhead costs, employee relations, billing, and advertising. The biggest ethical challenge is balancing the roles of being a

professional and an **entrepreneur**, as these roles are not always congruent. For example, a solo practitioner might want to introduce new therapies but faces challenges due to time and role constraints.

The independent practitioner is in charge of:

- Managing and planning the practice.
 - Documentation and record-keeping.
 - Dealing with third parties.
 - Protecting confidentiality.
 - Managing finances.
 - Staff training and office policies.
 - Advertising and marketing.
-

2. Issues in Independent Practice

Independent practitioners may face unique ethical problems due to the absence of peer collaborators.

- **Lack of Peer Collaboration:** Without colleagues, a solo practitioner may not receive professional guidance, which can hinder their growth. In a hospital setting, complex cases are discussed among professionals to find better solutions. If an independent psychologist makes an error in diagnosis, there may be no one to correct it.
- **Absence of Paid Leave:** Solo practitioners generally don't have the luxury of paid vacations or sick days. A therapist who was disabled for three months after a car accident had no one to cover his clients, which could be a serious ethical oversight depending on their needs.
- **Troubled Clients:** Independent practitioners are more susceptible to problems with emotionally troubled clients, such as those who don't pay bills or miss appointments.
- **Misleading Clients:** A new professional might avoid advertising their clinic to save money, which can mislead clients. A case example involved a woman who mistakenly went to a therapist's office thinking it was a gynecologist's office.

Lesson 38: Ethics Regarding Scholarly Publications

1. Challenges in Scholarly Publication

Honesty and competence are essential for the advancement of knowledge through scholarly publication. However, the scientific record can be damaged by violations ranging from minor to severe.

- **Key Issues:**
 - **Plagiarism and Misconduct:** Scandals involving plagiarism and other scientific misconduct are a major concern.
 - **Conflicts of Interest:** Researchers may have conflicts of interest that compromise their objectivity.
 - **Reluctance to Acknowledge Flaws:** Researchers might be unwilling to accept flaws in their work, especially after investing significant money in a project.
- **External Pressures:** Researchers also face external forces that can act against them.
 - **Political Suppression:** Scientific findings may be suppressed or misrepresented for political gain, such as ignoring evidence of global warming.
 - **Funding Influence:** Government agencies that fund research may intervene in the peer-review process, allowing political agendas to influence scientific outcomes.
 - **Attacks on Contradictory Facts:** Well-researched, peer-reviewed facts that contradict what people want to believe may be harshly attacked or suppressed.

2. Publication Outlets and Competition

Knowledge is shared through scholarly books, journals, and electronic platforms. The primary purpose is to disseminate useful discoveries as soon as possible.

- **Motivations for Publishing:**

- **Career Advancement:** Publication credits are often required for admission to graduate school, postdoctoral appointments, job acquisition, promotions, or securing grant funding.
- **Professional Status:** Publications can elevate a researcher's status among peers and bring widespread recognition.

- **Negative Consequences of Competition:**

- **Quantity over Quality:** A focus on the number of publications can lead researchers to pursue projects that can be completed quickly rather than more noteworthy, in-depth studies.
- **Inflated Importance:** Authors might exaggerate the significance of their work and fail to discuss study limitations.
- **Predatory Journals:** There is an increasing number of predatory journals, and researchers must be skeptical and carefully examine publishers to avoid them.

3. Publishing Case Studies & Narratives

Many mental health professionals publish qualitative analyses of client stories and experiences. Publishing this material is not inherently unethical. However, it raises issues of informed consent, confidentiality, and privacy invasion.

- **Protecting Privacy:**

- **Disguising Information:** Case studies should be disguised by removing all identifying information. However, in writing about a single person, the risk of them being identified remains present.

- **Obtaining Authorization:** It is a wiser course of action to seek a current or former client's specific authorization for the actual content to be published. Even with disguised contexts, courts could find in favor of an offended client if detailed descriptions are published.
-

4. Handling Conflicts of Interest

Conflicts of interest can arise when a researcher, author, editor, or peer reviewer has a relationship (personal or financial) that could affect their objectivity or influence their decisions.

- **Sources of Conflict:**
 - **Financial relationships:** Including direct employment, grants, or stock options.
 - **Personal relationships:** Such as rivalries or bias.
 - **Intellectual beliefs:** Moral convictions or personal beliefs that can influence scientific opinions.
 - **Academic competition:** Biased judgments due to direct or indirect competition.
 - **Managing Conflicts:**
 - Finding oneself in such a situation is not necessarily unethical. Stakeholders should take steps to avoid conflicts of interest, including:
 - **Disclosure:** Disclosing all interests so stakeholders are aware.
 - **Monitoring:** Monitoring research and results for transparency and integrity.
 - **Removal:** Removing individuals who may be biased from important processes like data interpretation or review.
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5. Managing Research Data

Data are the most important outputs of research and are used to accept, reject, or frame hypotheses.

- **Key Aspects of Data Management:**

- **Data ownership:** This involves the legal rights to the data, which may belong to funders, institutions, or principal investigators.
 - **Data collection:** This requires consistent, quality-controlled data collection with appropriate authorization and methods.
 - **Data storage:** Data must be protected from damage, theft, or loss to allow for re-analysis, rechecking of findings, and prioritization of future research.
 - **Data sharing:** Decisions must be made about what data to share and with whom, while maintaining the privacy and confidentiality of human subjects.
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6. Assigning Authorship

Assigning authorship gives credit and acknowledges contributions to research. It also implies accountability and responsibility for the published work. The International Committee of Medical Journal Editors (ICMJE) requires authors to meet four criteria:

1. **Substantial Contribution:** Contributing to the conception, design, data acquisition, analysis, or interpretation.
2. **Drafting and Revising:** Drafting the work or critically revising it for intellectual content.
3. **Final Approval:** Giving final approval for the version to be published.
4. **Accountability:** Agreeing to be accountable for all aspects of the work and ensuring that questions about accuracy or integrity are resolved.

- **Authorship Issues:**

- **Guest Authorship:** An author is listed without having contributed, solely for their credentials.
- **Gift Authorship:** An author is associated with the work but does not meet the ICMJE criteria.
- **Ghost Authorship:** An author who should have been recognized is excluded from the list.
- **Order and Omissions:** Disputes can arise over the order of authors' names or the omission of co-authors who were part of the study.

Lesson 39: Research Misconduct

1. Defining Research Misconduct

Research misconduct is defined as the "three cardinal sins" of research conduct:

fabrication, falsification, or plagiarism. These behaviors are observed in the design, execution, review, or reporting of research findings. Errors that are unintentional or situations arising from honest differences of opinion are not considered scientific misconduct.

- **Falsification:** Misrepresenting research by changing data, results, equipment, methods, or materials.
- **Fabrication:** Reporting false or made-up data, results, or research outputs.
- **Plagiarism:** Presenting someone else's ideas, work, or words without giving them appropriate credit.

2. Plagiarism and Unfair Use

Plagiarism is condemned in most ethics codes and can range from careless paraphrasing to intentionally copying large sections of work without citation.

- **Important Issues:**

- **Unintentional Plagiarism:** Younger professionals, accustomed to accessing information online as students, may be desensitized to the seriousness of plagiarism.
- **Source of Discovery:** The most common source for uncovering major acts of plagiarism isn't other scholars, but students conducting literature reviews.
- **Fair Use:** The "fair use" violation is related to plagiarism but is often unintentional. It allows scholarly writings to quote short, properly cited sections of a work without explicit permission.

- **Types of Plagiarism:**

- **Complete Plagiarism:** Submitting someone else's work under your name.
- **Source-Based Plagiarism:** Citing a source that is incorrect or doesn't exist. It can also occur when an author cites a primary source but not the secondary source where they found the information.
- **Verbatim Plagiarism:** Copying word-for-word without quoting and citing.
- **Self-Plagiarism:** Reusing significant portions of your own previously published work without attribution.
- **Paraphrasing Plagiarism:** Using someone else's writing with minor changes and presenting it as your own.
- **Mosaic/Patchwork Plagiarism:** Interlaying phrases or text from another source within your own work.

- **Redundant and Salami Publication:**

- **Redundant Publication:** Submitting the same (or slightly altered) paper to different significant outlets without the editors' permission.

This wastes reviewers' time, takes up journal space, and misleads readers.

- **Salami Publication:** Slicing a single research study into smaller, seemingly independent articles to increase the number of publications. This can distort the perceived value of the findings and waste editorial resources.
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3. Data and Image Manipulation

Data manipulation involves forging, changing, or presenting scientific data unprofessionally. Researchers may manipulate data to avoid further experiments, make the process easier, or validate a hypothesis. This can lead to false theories, distorted perceptions, and risky experiments.

- **Image Manipulation:** This has become a growing type of misconduct, with nearly 4% of papers showing suspicious alterations. Image manipulation is classified into three categories:
 - **Acceptable:** Formatting an image for publication requirements.
 - **Inappropriate:** Modifying an image without changing the interpretation of the results, such as removing background noise.
 - **Fraudulent:** Modifying an image in a way that affects the interpretation of the results, such as removing a band from a negative control lane.
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4. Research Reproducibility

Reproducibility is important for validating findings. It is defined as a researcher's ability to duplicate a phenomenon even when experimental conditions are varied. This is different from replicability, which is obtaining the same results under the same conditions. To make research reproducible, a researcher should:

- Write detailed, easy-to-understand experimental protocols.
- Share research outputs in an open-access repository.
- Perform experiments with variations to increase the robustness of the findings.
- Avoid data fabrication or manipulation. It is important to note that reproducible research is not always correct and can still have flaws, such as false positives, poor study design, or missed confounding variables.

Lesson 40: Cultural & Demographic Issues in Research

1. Cultural and Demographic Issues

Social and behavioral researchers often study populations that are different from themselves in substantial ways, such as age, race, religion, sex, or socioeconomic status. Researchers who are unaware of a study group's customs or characteristics, or who have biases, are more likely to conduct poorly designed studies. This can lead to incorrect or misleading results and cause social harm to the population being studied. For instance, a psychological test that is valid for one culture may not be valid for another. This is why the development of indigenous tests is encouraged in countries like Pakistan. It's crucial to understand a community's way of life to avoid flawed study designs and biased results.

2. Ethical Issues in Vulnerable Populations

Research ethics standards apply best to competent adults with a sense of autonomy who can make fully informed decisions. However, many research populations are vulnerable and cannot exercise full self-determination. These groups include children, prisoners, pregnant women, the mentally

disabled, and economically or educationally disadvantaged persons. Special attention is required when working with these populations because more trauma-related questioning can aggravate their symptoms. The infamous "Monster Study," which subjected orphan children to belittling to induce speech imperfections, and the Tuskegee study, which left poor black men with syphilis untreated, are terrible examples of disregard for the welfare of vulnerable participants.

3. Balancing Benefits & Risks

Calculating the risk/benefit ratio is a crucial, though often tricky, part of research. In general, a substantial and vital project may have a higher acceptable risk, especially if participants might benefit from the intervention. If risks are low and benefits are high, the research is likely to be approved. Conversely, if risks are high and benefits are low, approval is unlikely unless there are no other ways to help individuals in irreversible states. When both risks and benefits are moderate, the decision becomes more complex, and consent forms that participants can clearly understand become critical.

4. Assessing Risks

Risk assessment is the probability that unwanted harms will occur from participating in a study. Risks are evaluated based on what the harms could be, to whom, their seriousness, and their reversibility. The six types of potential risks in research with human participants are physical, psychological, social, economic, legal, and dignitary. Most social and behavioral research has non-existent or trivial risks. However, serious risks can include invasion of privacy, breach of confidentiality, and lingering stress. A complicating problem is that the degree of risk for new techniques may be unknown.

5. Assessing Benefits

Potential benefits from research are often difficult to estimate accurately because a procedure's benefits are, by definition, not yet known. In social and behavioral research, a benefit may exist primarily from the researcher's perspective. For example, a researcher might believe that teaching children assertiveness is beneficial, while a critic might argue it could lead to them being perceived negatively by adults. The "benefit test" has also been debated on who or what benefits, with some arguing that the benefit should apply strictly to the participants themselves.

Lesson 41: Research Outside Traditional Settings & Self-Deception

1. Research Outside Traditional Settings

Much of the ethical guidance for research applies to studies conducted within academic institutions or hospitals, where participants come to the researchers. However, when data is collected in schools, public places, or private homes, new ethical dilemmas can arise.

- **Informed Consent:** In field research, participants may not be aware they are being observed, which limits the need for advance voluntary and informed consent. For example, a researcher might observe people in a naturalistic setting like a park without any manipulation. In other cases, participants might be deceived to study their reactions, but debriefing is required afterward.
- **Confidentiality & Privacy:** In naturalistic observations of public behavior, confidentiality issues are minimal if identifying information cannot be linked to the people being observed. However, when using visual or audio recordings, it is necessary to disguise or remove the possibility of recognition.

2. Research & Multiple-Role Relationships

Dependent relationships are characterized by a power imbalance between the researcher and the participant, which can compromise the participant's ability to give free consent or withdraw from a study.

- **Examples:** A professor asking their students to participate in a research project as an assignment. A researcher's current or previous clients participating in a study, where their participation could jeopardize therapeutic services.
 - **Safeguards:** Researchers should ensure appropriate consent is obtained from "gatekeepers" like school principals or legal guardians. Current or previous clients should generally not be included in research unless the benefits are likely and not available otherwise. In situations with a large power differential, special safeguards, such as a participant advocate, should be introduced to ensure participants can withdraw without harm.
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3. Making Ethical Decisions & Taking Action

Mental health professionals have a proactive role to play when a colleague or student engages in a questionable act that could cause harm or lower public trust.

- **Cultural Competency:** When working with diverse clients whose values conflict with those of Western civilizations, therapists need specialized knowledge. Knapp and VandeCreek (2007) proposed a "**Soft Universalistic**" approach, which recognizes that most cultures share basic values, even if they are expressed differently.
 - **Practitioner's Role:** Therapists should focus on principle ethics (e.g., respect and compassion) rather than specific actions. They should reflect on the impact of their own beliefs and practices and avoid being judgmental or biased.
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4. Self-Deception

Self-deception is seeing the world the way we wish it to be, rather than how it is. People use their hopes, desires, and prejudices to construct their view of the world.

- **Characteristics:** Humans tend to sample information that is positive or consistent with their ideology, and avoid inconsistent or negative elements.
- **Types:** Self-deceptions can be individual, but they can also be **cultural** when they are widely shared. For example, the belief that supernatural beings make the world work was once a widespread cultural self-deception.
- **"Fair Use" Standards:** The violation of "fair use" standards is related to plagiarism but is more likely to occur unintentionally. Scholarly writings are typically allowed to quote short, properly cited sections without explicit permission from the copyright holder.

Lesson 42: Red Flags

1. Understanding Red Flags

A "red flag" is a warning sign that indicates a potential for danger or harm. While not every warning sign is an ethical violation, its appearance requires careful consideration and an appropriate response. These warning signs are often missed due to factors like rationalization, high stress, incompetence, or carelessness. Processing critical information without full awareness makes professionals prone to acting on influences they don't fully perceive.

2. Red Flags in the Therapeutic Relationship

Several red flags can arise within the therapeutic relationship itself.

- **Desire for a Different Relationship:** This includes a therapist disclosing irrelevant personal details or frequently thinking about a client outside of sessions.
 - **Personal Ambition and Financial Gain:** It is unethical to view a client as a way to advance your career or fulfill personal needs. It's also a red flag to accept clients when your training is insufficient, fail to refer them when they are not benefiting, or ask them for favors or loans.
 - **Enhancing One's Own Self-Esteem:** A therapist should not rely on a client's praise to feel good about themselves. Believing you are the only one who can help a particular client or indulging in "rescue fantasies" is also unethical. A therapist should act as a guide, not a savior, and avoid making a client dependent on them. Taking full credit for a client's achievements is also a red flag.
 - **Expecting Clients to Fulfill Personal Needs:** Anticipating favors from a client (e.g., getting a better deal from their business) is highly discouraged. Viewing clients as central people in your life is also a red flag, as it increases the risk of transference or countertransference.
 - **Fear of Rejection or Termination:** It's unethical to encourage a client's dependence or feel dread when a client might quit therapy. Resisting termination despite clinical indicators is discouraged as it hinders client autonomy. Frequently allowing sessions to go over the allotted time is also damaging to the relationship.
 - **Negative Feelings Toward a Client:** Feeling bitter about a noncompliant client, anxious about an appointment, or put off by a client who resembles someone you dislike are all red flags to be avoided.
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3. General Red Flags

Beyond the therapeutic relationship, other red flags include:

- Allowing a problematic relationship with a colleague to worsen.

- Being ignorant of the ethical standards of your profession or the emergency resources available in your community.
- Feeling uncomfortable discussing looming red flags with a trusted colleague for fear of being judged negatively.

Lesson 43: Making Ethical Decisions

1. Role-Blending Decisions

Role blending is responsible for a significant number of ineffective or careless decisions made by therapists. Role conflict occurs when the expectations of one role are incompatible with another. Ethical decisions aim to minimize the misuse and exploitation of a therapist's power, not to avoid all boundary violations.

- **Guidelines for Assessing Harm:** Three factors can help assess the potential harm from role blending:
 1. **Incompatible Expectations:** As expectations between the professional and the client become more incompatible, the potential for harm increases.
 2. **Divergent Obligations:** When obligations associated with the roles diverge, the risk of losing objectivity and having divided loyalties rises.
 3. **Power Imbalance:** When the therapist's power and prestige exceed the client's, the possibility of exploitation is heightened.
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2. Making Decisions with Lead Time

Complex and difficult decisions require active planning, as a "trust-your-gut" approach can be dangerous. When ethical dilemmas arise and there is sufficient time to plan, the best outcomes are more likely if a few key factors are present:

- **Sufficient Time:** There should be enough time to gather all important information to explore options, consult with others, and plan for intervention and follow-up.
 - **Involvement:** There should be an opportunity to involve all relevant parties in the decision-making process.
 - **Objectivity:** The decision should be made under low stress and with a mindset that maximizes objectivity.
 - **Continuous Evaluation:** The process should include a continuous evaluation that allows for modifications to satisfactorily resolve the dilemma.
 - **Decision-Making Strategy:** Mental health professionals should internalize a decision-making strategy to help them cope with ethical matters as they arise.
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3. Suggested Decision-Making Strategy

Rest (1982) suggested that implementing a plan of action involves several important factors:

- **Sequence of Actions:** Figuring out the specific, concrete actions to be taken.
- **Overcoming Obstacles:** Working around impediments and unexpected difficulties.
- **Perseverance:** Overcoming fatigue and frustration.
- **Goal Orientation:** Keeping the eventual goals in sight.
- **Focus:** Resisting distractions and other allurements.

The time it takes to make a decision can vary from seconds to days or weeks, depending on the complexity of the situation.

- **Steps for Ethical Decision-Making:**

1. **Determine if it's an ethical matter:** The first step is to confirm that the situation actually involves an ethical issue, as the line between poor etiquette and unethical behavior can be blurred.
2. **Consult available guidelines:** Refer to existing guidelines, such as research evidence or case studies, that might apply to the situation for a possible resolution.

Lesson 44: Ethical Decision Making in Crisis

1. Further Decision-Making Strategies

Ethical decision-making is a complicated process influenced by our perceptions and values. When facing an ethical dilemma, it's beneficial to have a structured approach.

- **Step 3: Pause to Consider All Factors**
 - Assess situational factors that might influence your decision.
 - These factors can include the seriousness of the offense, whether others are aware of it, and who might be harmed.
- **Step 4: Consult a Trusted Colleague**
 - Seek input from others, especially experts in the field, as this can be very beneficial.
- **Step 5: Evaluate All Parties**
 - Assess the rights, responsibilities, and vulnerabilities of everyone affected.
 - Failing to consider a stakeholder's right to confidentiality or informed consent can lead to a flawed decision.
- **Step 6: Generate Alternative Decisions**
 - Brainstorm all possible options without focusing on their feasibility, including alternatives that seem risky or inappropriate.

- Also, consider the alternative of not making a decision or doing nothing at all.
 - **Step 7: Enumerate the Consequences**
 - Identify potential consequences for each decision, such as psychological and social costs, short-term and long-term effects, resource limitations, risks (e.g., violating individual rights), and benefits.
 - **Step 8: Make the Decision**
 - If you have completed the previous steps conscientiously, you should have all the information you need to make an informed decision.
 - **Step 9: Implement the Decision**
 - This is a critical step, even if the course of action seems clear.
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2. Behavioral Emergencies vs. Crises

Therapists may feel anxious and make unsatisfactory or self-serving decisions when they don't have time to prepare carefully. Distinguishing between a behavioral emergency and a crisis is important for decision-making.

- **Behavioral Emergency**
 - A behavioral emergency requires an **immediate response and intervention** to prevent possible harm.
 - Examples include suicidal or violent behavior.
 - The therapist must first evaluate the client's status, then intervene to reduce the risk of harm, which can range from nonjudgmental listening to ordering inpatient hospitalization.
 - Finally, a plan for the next steps must be created.
- **Crises**
 - A crisis is an **external event** that causes a loss of psychological

equilibrium and coping difficulty.

- Examples include a job loss or trauma.
 - In a crisis, individuals may seek or welcome assistance.
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3. Clients at Risk for Emergencies & Crises

Some clients wait until their situation is urgent before seeking help, forcing therapists to make critical judgments with incomplete information and before a professional relationship has been formed.

- **Assessing Suicide Risk:** Therapists have a heavy responsibility when a client may pose a suicide risk. It's essential for all psychotherapists to be well-versed in the clues, which include:
 - A verbal statement of intent.
 - Suicidal ideation.
 - A history of past attempts.
 - A precipitating event.
 - Deterioration in social or vocational functioning.
 - A plan of action.
 - Expressed feelings of hopelessness and despair.
 - **Documentation:** When working with potentially suicidal clients, therapists must carefully document their concerns and decisions. These records are critical for a defense if a therapist is ever sued.
 - **Specific Diagnoses:** Clients with certain diagnoses, such as **borderline personality disorder**, are exceptionally prone to crises due to emotional lability, impulsivity, and difficult relationship histories.
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4. Preparing in Advance for Crises

While crisis management is a broad topic, a few suggestions can help therapists prepare for difficult decisions under tight time constraints.

- **Know Emergency Resources:** Keep an easily accessible list of names, numbers, and descriptions of community emergency services.
 - **Consult with Colleagues:** Form an alliance with other mental health professionals who agree to be available for consultation during emergencies. Ideally, this group should include someone with experience in emergency care.
 - **Know the Laws:** Be familiar with state and local laws regarding mandated reporting and commitment procedures.
 - **Locate an Attorney:** Find an attorney knowledgeable about legal matters relevant to your practice.
 - **Seek Continuing Education:** Sharpen your knowledge about crises that may arise by taking courses in crisis counseling or first aid.
 - **Define Competence:** Consciously define your areas of competence and practice only within those confines.
 - **Monitor Client Relationships:** Carefully monitor the therapeutic relationship, as miscalculations can lead to unanticipated outcomes.
 - **Document Everything:** Never rely on your memory alone. Document all emergency or crisis events, including your decisions and the rationale behind them.
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5. Crises in the Therapist's Life

Therapists can also experience personal crises that affect their professional commitments.

- **Example Case:** A counselor, grieving the drowning of her granddaughter, was unresponsive and distracted during a session with a client. The client, feeling ignored, complained, and the therapist disclosed her personal tragedy. The counselor's handling of her personal grief was poor, as it left the client with mixed feelings of embarrassment and sympathy.

- **Ethical Handling:** The therapist should have considered canceling appointments or referring urgent cases to a backup therapist, explaining that she needed time to deal with a "pressing family matter". It would also be appropriate for the therapist to send the client a note apologizing for the discomfort caused by the situation.

Lesson 45: Dealing with Other's Unethical Behavior

1. Understanding Unethical Behavior

A surprisingly high number of scientists and advanced students are aware of scientific misconduct by their peers, but most fail to report it. Even good people can act unethically without realizing it. It's often easier to confront someone who you believe acted without awareness, as the goal becomes education rather than accusation.

An individual's decision to act unethically depends on three factors:

1. **Moral Assessment:** The individual's personal judgment of the act.
2. **Benefits:** The perceived advantages of acting unethically.
3. **Perceived Risk:** The individual's perceived risk of getting caught.

If an individual's own values prevent them from moving forward, any thought of an unethical act is quickly reversed. However, a strong intention to carry through is generated when a person rationalizes the act, predicts sufficient advantage, and perceives a low risk of detection. Peers and colleagues are in the best position to intervene, minimize harm, and prevent the behavior from recurring.

Three types of interventions can be helpful in dealing with others' unethical behavior:

- **Reminding:** Using subtle cues to increase awareness of moral behavior and decrease the ability to justify dishonesty.
 - **Visibility:** Restricting anonymity and encouraging peer monitoring and responsible norms.
 - **Self-engagement:** Increasing an individual's motivation to maintain a positive self-image and generate a personal commitment to act morally.
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2. Informal Confrontation

An attractive feature of informal peer monitoring is that it can solve a problem and save a colleague from a more formal correctional process simultaneously.

Pre-confrontation Preparation

Before confronting a colleague, consider these hints:

- **Identify the Ethical Principle:** Pinpoint the relevant ethical principle or law that applies to the suspected breach.
- **Reflect on Your Motives:** Thoughtfully consider your own reasons for engaging in—or avoiding—a confrontation.
- **Consider Cultural Issues:** Assess any cultural factors that might help you better understand the situation.
- **Assess the Evidence:** Evaluate the strength of your evidence. Ethical infractions often lack tangible exhibits. Categorize your information source:
 - Direct observation of unethical behavior.
 - Direct disclosure from the colleague about a violation.
 - Direct observation of suspicious, but not clearly interpretable, behavior.
 - A credible secondhand report.
 - Casual gossip.

- **Consult a Colleague:** Consult with a trusted colleague who has demonstrated sensitivity to ethical issues.

The Confrontation

When you decide to confront, these points can facilitate the process:

- **Schedule a Meeting:** Schedule the meeting in advance in a professional setting, like an office.
- **Set the Tone:** Aim for a constructive and educative session. View yourself as an ally, not an accuser or judge.
- **Remain Calm:** Stay calm and self-confident. The colleague may become emotional, so remain as nonthreatening as possible.
- **Describe Your Obligation:** Clearly state your ethical obligation and the relevant principle that prompted your intervention. Avoid playing detective with leading questions, which only leads to defensiveness.
- **Allow for Explanation:** Give the colleague ample time to explain their position in detail. Be patient, as they may become flustered or defensive.
- **Address Abusive Behavior:** If the colleague becomes abusive, attempt to steer them to a more constructive state, possibly by including another person or pressing formal charges.

3. Informal vs. Formal Resolution

Ethics codes give members the option to decide if dealing with a matter directly is appropriate, especially given reports of harassment and intimidation.

- **When to take Formal Action:** If an informal solution seems unlikely and substantial harm has occurred or is likely to occur, formal action (e.g., contacting a licensing board or ethics committee) should be taken.

- **When Informal Intervention is Ineffective:** Informal intervention won't resolve problems related to a colleague's general incompetence from insufficient training or emotional impairment. Such individuals often lack insight into their shortcomings and could cause significant harm.
 - **Anonymous Complaints:** You should not approach colleagues with charges from unseen accusers, as this violates due process. Furthermore, alleged violators often know who their accusers are anyway.
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4. Many Faces of Ethics Codes

Ethics codes for mental health professionals promote acting in the best interests of consumers. These codes serve multiple functions:

- They are public relations documents that enhance public confidence in the profession.
 - They spell out the principles that morally responsible members are expected to follow.
 - They clarify the proper use of skills and provide general guides for decision-making.
 - They assist in educating new professionals.
 - They define the rules for judging those whose actions are called into question.
 - They serve as tools for licensing boards and civil litigants.
 - They give consumers an additional layer of protection.
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5. Ethics Complaints Not Pursued

Ethics committees cannot pursue every complaint. The most common reasons the APA declines to consider a complaint include:

- **No Provision in the Code:** The code can't detail every inappropriate act, so some complaints fall outside its scope.
- **Not the Appropriate Mediator:** The committee may refuse to process complaints when it's clear they can't contribute to a solution, such as in cases of inter-professional political disputes.
- **Respondent is Not a Member:** Professional associations are voluntary, and their ethics committees only have jurisdiction over current members.
- **Against Groups or Agencies:** Ethics committees are not set up to deal with complaints against an organization, corporation, or group, only named individuals.
- **Anonymous Complaints:** Complaints are not pursued when they are anonymous, often due to a fear of retribution.
- **Improper Complaints:** Complaints judged to be frivolous and intended to harm or harass someone rather than to protect the public are not pursued.
- **Beyond Time Limits:** Complaints must be filed within a specified time period after the alleged violation occurred